

# Manual Handling Guide for Caregivers

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This Manual Handling guide supports safe moving and handling of clients in their homes. It supplements certified Manual Handling and Patient Moving & Handling training and must be read alongside each client's care plan. Refresh practical training at least every three years, or sooner after an incident or equipment change.

## TILE Risk Assessment Framework

### TILE Risk Assessment



Before every transfer or repositioning task, assess Task, Individual, Load, and Environment. If any factor presents unacceptable risk, stop, replan, request a second carer, or defer the task and notify the office.

### 1. TILE Checklist (Detailed)

| Factor      | Questions to Ask                       | Red Flags                        | Action                         |
|-------------|----------------------------------------|----------------------------------|--------------------------------|
| Task        | Necessary? Can task wait?              | Unplanned urgent move            | Call office; reassess plan     |
| Individual  | Am I fit? Trained? Client cooperative? | Pain, dizziness, confusion       | Do not proceed; medical review |
| Load        | Weight, grip, unpredictability?        | Client cannot weight-bear safely | Use hoist; dual-up             |
| Environment | Space, lighting, floor, clutter?       | Wet floor, narrow stairs, pets   | Clear path or postpone         |

### 2. Before Any Transfer

- Read the mobility section of the care plan and any recent addendum.
- Confirm equipment on site: hoist, sling, slide sheet, transfer belt, wheelchair, zimmer.
- Inspect slings for wear, tears, or incorrect labelling; do not use damaged equipment.
- Explain the move to the client; obtain verbal consent; allow adequate time.
- Ensure brakes applied on bed, wheelchair, and hoist; spread hoist legs fully.
- Remove trailing cables, rugs, and clutter from the transfer path where safe.

### 3. Equipment Reference

| Equipment     | Indication                                      | Key Steps                     | Never Do                                    |
|---------------|-------------------------------------------------|-------------------------------|---------------------------------------------|
| Transfer belt | Care plan allows; client partial weight-bearing | Fit snug; guide don't lift    | Lift client by belt alone                   |
| Slide sheet   | Repositioning in bed                            | Roll client safely; use sheet | Drag client on bare mattress                |
| Hoist + sling | Non-weight-bearing / plan requires              | Correct sling size & loops    | Use wrong sling or solo if dual-up required |

| Equipment      | Indication                    | Key Steps                         | Never Do                                       |
|----------------|-------------------------------|-----------------------------------|------------------------------------------------|
| Zimmer / frame | Sit-to-stand with supervision | Brakes off only when moving frame | Leave client unsupported on frame              |
| Profiling bed  | Raise to working height       | Lock wheels before transfer       | Transfer with bed at floor level unnecessarily |

## 4. Technique Summaries

### Sit-to-stand

Client sits forward, feet flat, hands on armrests. Carer supports at trunk or uses belt as plan allows. Client pushes up; carer guides upward without pulling under arms. Pause if dizziness reported. Position zimmer or frame before release.

### Bed repositioning

Use slide sheet with at least one other person if plan requires. Log roll where appropriate. Never drag client across sheets. Raise bed to reduce stooping. Pillows and wedges per plan.

### Hoist transfer

Attach sling per manufacturer colour/size code. Confirm loops on correct hooks. Lift slowly while monitoring client comfort. Lower gently; do not leave client suspended unnecessarily. Two carers when plan specifies dual-up.

## 5. When NOT to Proceed

| Situation                  | Document                            | Notify                       |
|----------------------------|-------------------------------------|------------------------------|
| Client refuses             | Note refusal & alternatives offered | Office same visit            |
| Equipment missing/broken   | Equipment fault form                | Office immediately           |
| Client unwell / post-fall  | Incident or near-miss report        | Office + emergency if needed |
| Solo when dual-up required | Do not attempt                      | Office for cover             |
| Unsafe environment         | Photo if policy allows              | Office; postpone task        |

## 6. After Care & Maintenance

Record transfer difficulties, equipment faults, or mobility changes in visit notes. Report defects the same day. Slings and hoists must be serviced per manufacturer schedule. Store slings off floor, labelled, and matched to client where personalised.

## 7. Common Injuries & Prevention

| Injury Type        | Common Cause           | Prevention                                  |
|--------------------|------------------------|---------------------------------------------|
| Lower back strain  | Twisting while lifting | Face client; pivot feet; use equipment      |
| Shoulder injury    | Pulling under arms     | Use belt/hoist; encourage client effort     |
| Client fall        | Rushing; poor footwear | Stable footwear; clear path; pause if dizzy |
| Skin tear          | Dragging on sheets     | Use slide sheet; lift don't drag            |
| Staff pinch injury | Hoist spreader bar     | Keep hands clear; communicate with partner  |

## 8. Room-Specific Guidance

### Bathroom transfers

Non-slip mats must be in place. Check water temperature before showering. Use shower chair and grab rails as per

plan. Never leave client unattended on wet floor. Ensure hoist reach is adequate in small bathrooms before starting; request equipment review if not.

Stairs

Stair transfers require explicit care plan authorisation and usually dual carers. Ascend behind client if using zimmer; descend in front. One step at a time. Good lighting essential. If client fatigues mid-stair, stop on landing, make safe, and call office.

Vehicle transfers

Only perform if plan specifies and training received. Park on level ground. Apply brakes. Allow extra time. Use swivel cushion or slide board if supplied. Document any new difficulty.

9. Equipment Servicing Schedule (Example)

| Equipment     | Visual Check Each Visit   | Formal Service           | Action if Faulty               |
|---------------|---------------------------|--------------------------|--------------------------------|
| Hoist         | Battery, hooks, wheels    | Annual PAT + LOLER       | Quarantine; report same day    |
| Sling         | Stitching, labels, stains | Replace per manufacturer | Do not use; request spare      |
| Wheelchair    | Brakes, tyres, footrests  | 6-monthly service        | Tag out of use                 |
| Profiling bed | Controls, rails, wheels   | Annual                   | Manual crank if electric fails |

10. Case Study (Illustrative)

Client Patrick D. requires dual-up hoist transfer bed to chair each morning. On arrival, carer notices sling label does not match client size. Correct procedure: do not proceed; phone office; use spare labelled sling from cupboard if available and matches plan; document equipment check in notes. Attempting transfer with wrong sling is a serious breach.

Emergency: make safe, call 112/999 if injured, notify office, complete Incident Report within 24h.